TRT-GUY

THE MALE FERTILITY RESTORATION GUIDE FOR GUYS ON TRT/PEDS

By the TRT-Guy

IMPORTANT DISCLAIMER

I am not a doctor, and this is not medical advice. This guide explains my personal experience and is for educational purposes only.

01

HOW I RESTORED MY FERTILITY

MY EXPERIENCE

After years of PED use, my sperm count was zero.

I used a fertility-restoration protocol very similar to the one in this guide, brought my sperm back, and my wife and I conceived our first daughter without having to come off steroids (I was using higher doses back then).

This worked for me, and I want men in a similar position to understand the approach instead of automatically assuming their fertility is permanently gone.

BEFORE YOU START

> **Get a formal semen analysis before starting because you might have enough sperm to get your wife pregnant and this protocol will become irrelevant.**

Make sure you know where things are at before implementing anything, and this will allow you to compare and decide if the protocol is working.

02

THE HCG + HMG PROTOCOL

This schedule follows the incremental HCG + HMG protocol used by Finkel, Phillips, and Snyder.

TIMELINE	HCG	HMG	CHECKPOINT
Weeks 1-16	2,000 IU Monday, Wednesday, Friday	75 IU Monday, Wednesday, Friday	The drugs may be administered on the same days as separate injections. You can also alternate days if you wish.
Week 12	Continue 2,000 IU three times per week	Continue 75 IU three times per week	Primary semen analysis: concentration, total count, motility, morphology, and volume.
Weeks 17-32	Continue 2,000 IU Monday, Wednesday, Friday	Increase to 150 IU Monday, Wednesday, Friday	Continue semen testing approximately every 8-12 weeks.
When sperm returns	Review with clinician	Review with clinician	Keep trying naturally and consider sperm banking while numbers are favorable.

03

DO NOT CRASH YOURSELF WHILE TRYING TO CONCEIVE

You need sperm to conceive, but you also need libido, erections, energy, and a healthy relationship. In my opinion, deliberately crashing your testosterone and feeling terrible is not the ideal environment for trying to make a baby.

I didn't have to stop taking PEDs to conceive my first daughter, and I started with a sperm count of zero.

Personally, I wouldn't recommend removing TRT/PEDs since that will bring other issues, you will also experience hypogonadal symptoms which will make it harder to have sex, among other things like sadness, anxiety, poor sleep, etc.

04

EAT IN A SLIGHT CALORIE SURPLUS

This is not the time for an aggressive fat-loss phase. Sperm production is a resource-intensive biological process. I would rather give the body slightly more energy than it needs than ask it to restore fertility while dieting hard.

PRIORITY	WHAT I WOULD DO
Calories	Stay in a slight calorie surplus. The goal is enough energy for recovery without rapid, unnecessary fat gain.
Protein	Eat sufficient lean protein sources across the day to support recovery and general health.
Carbohydrates	Eat enough carbohydrates to support training, thyroid function, energy, and sexual performance.
Produce	Eat plenty of fruits and vegetables for micronutrients, fiber, and overall diet quality.
Fats	Include sufficient dietary fat from quality sources. Do not run an unnecessarily low-fat diet.
Food quality	Build most meals from nutritious whole foods. Supplements should add to the diet, not replace it.

05

INCREASE YOUR CHANCES OF CONCEPTION

HAVE SEX OFTEN

A protocol can improve the semen analysis, but conception still requires frequent intercourse.

My practical target is sex every day while trying to conceive. During the fertile window, once or twice per day is an option if both partners feel good and it does not turn the process into a chore.

Use ovulation strips or another reliable method to identify the fertile window. The highest-priority days are the two days before ovulation and the day of ovulation.

OPTIONAL ADDITIONS

ADDITION	HOW I WOULD USE IT	PURPOSE
Tadalafil	5-10 mg daily	More reliable erections and less performance pressure when intercourse needs to be frequent. This supports erectile function; it is not a direct libido drug.
Injectable L-carnitine	200-500 mg three times per week.	An optional addition intended to support sperm quality, cellular energy, and recovery.

OTHER BASICS THAT STILL MATTER

- Sleep enough to recover and maintain sexual function.
- Avoid smoking, recreational drugs, excessive alcohol, and frequent high-heat exposure to the testes.
- Keep moderate training in, but do not combine hard dieting, excessive cardio, poor sleep, and maximal training volume.

06

FREQUENTLY ASKED QUESTIONS

Do I need to come off TRT for this to work?

No.

Do I need to come off a steroid cycle?

No.

Can I guarantee that I will regain fertility?

No. I regained fertility and conceived my first daughter, but that is my personal outcome. The result depends on your fertility before PED use, age, duration of suppression, testicular function, genetics, medication quality, adherence, etc.

How long does it take for sperm to return?

Sperm production takes roughly 74 days, followed by additional maturation and transport. Some men show sperm by 8-12 weeks, while others need 4-6 months or longer.

Should I test at week 8 or wait until week 12?

Week 8 is an optional early look. Week 12 is the more useful first assessment because it allows approximately one sperm-production cycle. Use the same laboratory and follow its abstinence instructions so the results are easier to compare.

Do I need Clomid, enclomiphene, or Nolvadex as well?

They were not part of my protocol, therefore I can't recommend them.

How often should we have sex?

The more the better, especially during the fertile window.

When should my partner be evaluated?

If the female partner is 35 or older, has irregular cycles, or has a known reproductive issue, then it's probably a good idea to have her evaluated by a female fertility specialist to make sure everything is okay.

Should I freeze sperm when it comes back?

If you plan to have more kids later on, then it's a good idea to have some sperm frozen as a "back up" plan.

Can I mix hCG and hMG in the same syringe?

They may be scheduled on the same days, but you might need them in separate syringes due to the volume of each injection.

NEED HELP GETTING THE MOST OUT OF TRT/PEDS?

Through my one-on-one coaching program, I help men dial in their nutrition, training, cardiovascular fitness, daily habits, and accountability so they can build a lean, strong, athletic body without sacrificing their health.

Click here to apply.